

إسم المريض رباعياً : عزه صالح عباس عبدالله
المحافظة : الجيزة
الحى / المدينة : البدرشين
رقم البطاقة : 28502102102321 رقم التليفون : 01008689489
العمر(تاريخ الميلاد) : 10-02-1985 المهنة : ربه منزل

العنوان : +++
تاريخ تسجيل الإستمارة : 02-10-2026

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-
الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة
نفقات العلاج في حالة ثبوت غير ذلك
الإحصائي الاجتماعي
التوقيع: (-----) المقر بما فيه
التوقيع: (-----)

التعليق على التشخيص : -----

التشخيص النهائي :
 خطة العلاج :
 المستشفى :
 قسم :
 جهة العلاج :
 نفقات :
 ملاحظات :

رئيس
المجلس الطبي المتخصص

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Name: عزه صالح عباس عبدالله National ID: 28502T02102321	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
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تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد أنه
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المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Second Line Hormonal Treatment Plus Goserline
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التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين


 جمهورية مصر العربية
 بطاقة تحقيق الشخصية
 غزه
 صلاح عباس عبدالله
 قرية النورشين
 مركز النورشين - الجيزة
 ٢٨٥٠٢١٠٢١٠٢٣٢١
 ١٩٨٢/٠٦
 KP1373344


 ٢٠٢٤/٠٦ ٢٨٥٠٢١٠٢١٠٢٣٢١
 ربة منزل
 انثى
 حسن علي حسن عيسى
 البطاقة سارية حتى ٢٠٣٢/٠٦/٢٣
 ١





جذوه الإصدار: فرع الجيزة

تاريخ الإصدار: 2023-1-29

التي من يهمل الأمر

الهيئة العامة للتأمين الصحي تعمل من خلال قوانين منظمة للاحتفاع الخاضعين لقانون التأمين الاجتماعي رقم 79 لسنة 1975 المعدل بالقانون 25 لسنة 1977 "القوى العاملة والمعاشات والإرامل" وكذلك قانون 99 لسنة 1992 الخاص بطلاب المدارس وقانون رقم 86 لسنة 2012 "الأطفال دون السن المدرسي" وقانون رقم 23 لسنة 2012 الخاص بالمرأة المعيلة .
وطبقا لقاعدة البيانات المرسل من التأمينات الاجتماعية
وطبقا لقاعدة البيانات المرسل من الضمان الاجتماعي
والبحث تبين أن

السيد (ة) عزة صالح عيسى بطاقة رقم 28502102102321

وطبقا الى خطاب من التأمينات الاجتماعية بولي عدم الانتفاع
بمكتب من الفئات غير المستفعة بالتأمين الصحي



وشكر



تاريخ الطباعة: 2023-1-29

الإدارة مركز نظم المعلومات
بمكتب

مستند 381337



الحذاب صاري لمدة ثلاثة أشهر من تاريخ الإصدار إلا في حالة تغير حاله المواطن من قبل الهيئة القومية للتأمينات الاجتماعية أو
وزارة التضامن الاجتماعي أو وزارة التربية والتعليم

الإدارة المركزية لتكنولوجيا المعلومات و دعم القرار

Name: عزہ صالح عباس		
MR. : 1385		
Detailed Medical Report		
		السيد
تحيه طيبة وبعد,,		
	بشأن تقرير طبي للمريض	إيماء للطلب المقدم من
	رقم طبي:	بقسم:
نتشرف بأن نوضح فيما يلي تقريراً عن حالته		
	تاريخ وساعة الخروج :	تاريخ وساعة الدخول :
وبعد الكشف والفحوصات والمتابعة وجد انه The patient presented today for a routine visit and was administered zoladex 10.8 mg, arimidex, and supportive therapy for three months. The patient is a 41-year-old female with a positive family history of malignancy (grandmother). She is pre-menopausal and was diagnosed with right-side breast cancer, clinical stage CT4B, N2, M0. Histology revealed invasive ductal carcinoma grade II, estrogen receptor 8/8, progesterone receptor 8/8, HER-2 3+. She received neoadjuvant chemotherapy with TCHP, followed by right mastectomy and axillary dissection in October 2022. Post-operative radiation therapy was administered. She was started on anti-HER-2 therapy with KADCYLA and received tamoxifen plus zoladex in November 2022. Therapy was switched to arimidex plus zoladex in August 2024. The most recent CT of the chest in December 2025 showed a few bilateral pulmonary nodules, the largest measuring 5 mm.		
تحريرا في ,		
	التاريخ :	21-09-2026
	الوقت :	
	Reham Ahmed Fathy Ibrahim	الطبيب المعالج :
يعتمد ,		



مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

الهيئة العامة
للحساسية
والأمراض
مصر

MDT FORM

29/ 11 / 2022

Name	عزة صالح عباس	MRN ID	940 / 2022
Age	38 yrs	Phone	0111195119
Menopausal status	Premenopausal	Marital status	Married
NRS SCORE	2	Family history	-ve
BMI	obese 1	REASON OF MDT	Post-op
CO morbidity	-ve		5 No. 11.11.22

Clinical Data

BX:	IDC II			R	L
ER:	8/8	PR:	8/8	HER 2:	+3
				Ki67:	30%
Sonomammography:	-pre NACT scan				
Imaging Workup:	-CTs → Free Bone Scan → Free			TNM Previously CTub N2 Mo	

Recommendations: Post NACT + anti her2 (6TCHP + Post op
Rt mRM (IDG GII) LN 3/10
partial pathological response.
P11b N13/10

MDT OUTCOME & PLAN

For PMT / TDM, 14 cycles / adjuvant /
Adjuvant

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

29/11/2022

Patient Name : الأستاذة / عزة صالح عباس	Request No. : 13005575
Patient No. : 28502102102321	Request Date : 19/02/2022
Path No. : 464/22	Verification Date : 13/03/2022
Sex : Female	Financial Category : اورام دار السلام - مبادئه
Age : 37 Y	Department : Out Patient
Referring Doc. :	IHC No. :
	Registered by : Ali mostafa

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Right breast malignant looking solid mass.

Nature of the specimen: Tru-cut biopsy

Gross Examination:

- Procedure: Tru-cut biopsy (4 greyish white and fatty tissue cores, up to 1.5 cm long, totally submitted)
- Sections prepared from paraffin block for immune- staining using Bench Mark ULTRA system for ER, PR, HER2, ER clone (1E2), PR clone (SP1), HER-2 clone (4B5), & Ki67 clone (30-9).

Microscopic examination:

- Histologic type: Invasive duct carcinoma
- Histologic grade; (tubular formation + nuclear pleomorphism + Mitosis): GII (2+2+2)
- Ductal carcinoma in situ: Focal DCIS.

ER: Positive nuclear reaction in 95% of the tumor cells.

PR: Positive nuclear reaction in 95% of the tumor cells.

Her2: Strong continuous membranous staining (chicken wire) in more than 10% of tumor cells.

Ki67: Positive in 35% of tumor cells

Diagnosis:

Right breast mass, Tru-cut biopsy:

- Invasive duct carcinoma GII with solid DCIS

-ER: Positive, Score (8/8)

-PR: Positive, Score 8/8)

-Her2: Positive, Score 3+

-KI-67: Positive in 35% of tumor cells

Prof. Lubna Omar

Dr. Menna Atef

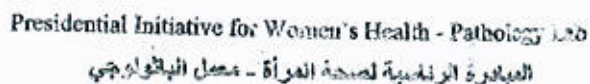
Verified by :

Dr.Marwa mahmoud

Printing Date : 13/03/2022 12:38:01

Page 1 of 2

Printed By : marwa mohamed youss



Patient Name	: الأستاذة / عزة صالح عباس عبدالله	Request No.	: 13003093
Patient No.	: 28502102102321	Request Date	: 24/10/2022
Path No.	: 3175/22	Verification Date	: 14/11/2022
Sex	: Female	Financial Category	: اورام دار السلام - مباحرة
Age	: 37 Y	Department	: Out Patient
Referring Doc	:	Registered by	: marwa mohamed youss

Pathology Report

Clinical Data:

Right breast cancer, post NACT.

Nature of the specimen: Right MRM specimen.

Gross Examination:

- Procedure: MRM specimen measured 20x19x9cm with skin covering measured 20x18cm, free of nipple and areola.
- Tumor extent
 - o Number: cut section revealed an ill defined grayish white tumour bed site.
 - o Size: measured 2.5x2x2cm.
 - o Skin involvement: unremarkable grossly.
- Distance from margins: 1.5cm from deep margin (least).
- Lymph nodes number and size: Axillary dissection revealed 10 lymph nodes up to 2x2cm.

Microscopic examination:

- Histological type: Remnants of Invasive duct carcinoma 9maximal diameter (1.2cm & 50% cellularity).
- Grade (differentiation+ nuclear pleomorphism + mitosis): GII (3+2+1).
- Angioinvasion: No.
- Ductal carcinoma in situ: No.
- Perineural invasion: No.
- Tumor extent
- o Skin involvement: Free.
- Lymph nodes involvement: positive micrometastasis in (3/10) (1ml).
- Resection Margins: free deep margin.
- Histologic response to therapy: Partial pathological response.

Diagnosis:

Verified by :
DR.Menna Nabil

Printing Date : 19/11/2022 11:34:45

Page 1 of 2

Printing By : manya mohamed youss



Patient Name: Gehad Saleh Abbas Abd Allah	Date : 07/09/2022
Exam : ultrasound BREAST (BOTH SIDE)	Age : 24 Year(s)
Medical ID: 1853	Pat. ID: 18605

BILATERAL BREASTS HIGH RESOLUTION ULTRASOUND

Indication:

FU

Report:

- Heterogeneous background echotexture (fibroglandular).
- Right two well-defined hypoechoic oval shaped solid lesions seen at:
 - 12:00 measuring 1.2x0.7 cm
 - 2:00 measuring 2.5x1.3 cm
- No left solid or cystic mass lesions.
- Normal skin thickness.
- Normal nipple-areola complex, with average caliber of the retro-areolar ducts
- Bilateral non-specific axillary LN, with preserved fatty hila and uniform cortical thickness.

Opinion:

- Right stationary probably benign looking solid lesions. For follow up (BIRADS 3)

Yours truly

Signature Dr. Yasmin Nada

07/09/2022 2:03 PM

- Right breast UIQ multifocal highly suspicious mass lesions with ipsilateral pathological axillary lymph nodes level I for histopathological correlation (BIRADS 5)
- Left breast lactational changes (BIRADS2)

Best regards,

Dr. Nehal Hamdey, FEBR. MSc

Patient name: عزة صالح عباس

Date: 16/02/2022

Combined bilateral breast mammography and complementary ultrasound

Patient is lactating

Technique

- Bilateral MLO and CC views
- Bilateral 3D MLO DBT (Digital Breast tomosynthesis)
- Bilateral breast ultrasound

Report:

- Heterogenous dense fibro glandular density that may obscure any lesion (ACR C).
- Right breast shows UIQ two spiculated dense mass lesion with microcalcifications with diffuse skin thickening.
- No suspicious solid mass lesion seen in left breast.
- **complementary ultrasound** right multiple irregular spiculated hypoechoic solid mass lesion with foci of microcalcifications seen at 2 o'clock zone C measuring about 22 x17mm and 23 x13mm with internal vascularity seen within and anterior ductal extension , the distance in between two masses about 23mm.
- small satellite lesion seen at 1 o'clock measuring about 17 x11mm.
- diffuse skin thickening reaching to 6.3mm.
- bilateral retro areolar and central dilated ducts with echogenic content. >>>> as lactating changes.
- left breast shows no suspicious masses or cysts.
- No left suspicious micro calcifications or architectural distortion detected.
- Normal left skin thickness, and nipple-areola complex with average caliber of the retro-areolar central ducts.
- Nonspecific left axillary LN.
- Right multiple pathological axillary lymph nodes with diffuse cortical thickening reaching to 7.8mm level I.
- Free supra and infra clavicular lymph nodes.

Opinion:



Patient Name: Azza Saleh Abbas Abd Allah	Date : 22/02/2024
Exam : MAMMOGRAPHY BOTH	Age : 39 Year(s)
Medical ID: 975/2022	

Left BREAST MAMMOGRAPHY WITH BILATERAL US

Indication:

Patient had right MRM

Technique

- Left breast MLO and CC views
- Bilateral ultrasound

Report:

- scattered fibroglandular densities of the left breast (ACR B)
- No spiculated masses or grouped microcalcifications.
- Complementary ultrasound also showed no left breast solid or cystic masses
- Normal skin thickness and nipple-areola complex, with average caliber of the retro-areolar ducts
- Nonspecific left axillary LN, with preserved fatty hila and uniform cortical thickness.
- Free right operative bed with no evidence of recurrent mass lesions
- Free right infra and supra clavicular regions.

Opinion:

- Right MRM with free operative bed with no evidence of recurrent mass lesions
- No left breast suspicious findings (BIRADS 2)

Yours truly,

Dr/Asmaa Zeinhom, Msc

Not Approved



مركز بؤية للتشخيص الميكرو
وعلاج سرطان السيدات

PATIENT NAME: AZZA SALEH ABASS ABDALLA	PATIENT ID # 2022185457
GENDER: Female	ORDER #: 15 0170,1510171
AGE: 37 y	DATE OF SERVICE: 06-Mar-2022 01:56 PM

Report: - all studies

**POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST,
ABDOMEN AND PELVIS**

1-CHEST:

- Diffuse right breast thickening is seen with multiple right breast masses the largest is seen measuring about 3.2 x 4.3 cm with multiple right axillary and sub-pectoral lymph nodes the largest measuring about 2.2 x 2.5 cm
- Prominent left axillary lymph nodes are noted.
- Mosaic attenuation of both lungs are noted.
- No sizable pulmonary nodules, masses, areas of consolidation or cavitory lesions.
- Right middle lobe parenchymal atelectatic band.
- No CT evidence of pathologically enlarged hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers.
- No pericardial effusion detected.

2- ABDOMEN AND PELVIS:

- The liver is enlarged in size with homogenous CT texture. No focal lesions or intra-hepatic or extra-hepatic biliary nodules dilatation..
- Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries.
- Normal appearance of the pancreas, spleen, kidneys, adrenals, aorta, I.V.C and the pelvic organs.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- No pathologically enlarged abdominal or pelvic lymph nodes.
- Multiple bilateral ovarian cysts are seen the largest is seen on the left measuring about 2 x 2.2 cm.

PATIENT NAME: AZZA SALEH ABASS ABDALLA	PATIENT ID #: 2022185457
GENDER: Female	ORDER #: 1538290
AGE: 37 y	DATE OF SERVICE: 28-Mar-2022 12:14 PM

Report:**99mTc-MDP BONE SCAN**

Desired Information :- skeletal survey.

Technique:

Delayed Whole body imaging in anterior and posterior projection performed 2 minutes & 2 hours after I.V administration of 20 mCi 99mTc-MDP :

FINDINGS:-

Degenerative changes at dorso- lumbar vertebrae.

Arthritic changes at shoulders.

Otherwise the rest of the surveyed skeleton is free from serious bone lesions.

Both kidneys are seen.

Conclusion:-

No definite evidence of metastatic osseous deposits,

Reported By,

Ass. Prof. Huda Fathy Elsayed

28-Mar-2022 04:07 PM